

Crying - Before 3 Months Old - Female (Child)

Algorithm 1488 | Infant (younger than 3 months) | Gender at birth: Female

Definition

- Safety-first unexplained crying assessment only for babies younger than 12 weeks. Crying with illness or another symptom requires assessment through the relevant symptom pathway; colic is considered only after serious illness, injury, feeding problems, and caregiver-safety concerns are excluded.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the baby's date of birth and exact age in weeks? This protocol applies only before 12 weeks of age.
2. [TEMPERATURE] What is the baby's measured temperature, how was it measured, and when?
3. [YES_NO] Is breathing normal, is colour normal, and is the baby alert and responding normally when awake?
4. [OPEN_TEXT] When did the baby last feed, how much compared with usual, and how many wet nappies have there been in the last 12 hours?
5. [YES_NO] Is there vomiting, a new rash, a swollen soft spot, pain when touched or moved, or any possibility of injury?
6. [YES_NO] Is the caregiver overwhelmed, angry, or afraid that they or someone else may shake or hurt the baby?
7. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Breathing Difficulty

Is breathing difficulty the main concern rather than crying or suspected colic?

Breathing difficulty belongs in the Breathing Difficulty pathway.

1. Disposition level 100

Is the baby not moving, very weak or floppy, difficult to wake, having a seizure, struggling to breathe, blue or grey, or is there immediate concern that anyone may shake or hurt the baby?

Reduced responsiveness, weakness, abnormal colour, breathing compromise, seizure, or immediate risk of inflicted injury can be life-threatening and requires emergency help.

1. Disposition level 70

Is the baby under 12 weeks with a temperature of 38 C or higher; a weak, high-pitched, or different cry; poor feeding; markedly fewer wet nappies or none for 12 hours; repeated or forceful vomiting; green vomit; a non-blanching rash; a swollen soft spot; pain when touched or moved; possible injury; abnormal behaviour; or nonstop inconsolable crying for 2 hours?

NICE classifies temperature 38 C or higher before 3 months as high risk. Feeding reduction, reduced urine, abnormal responsiveness, abnormal cry, vomiting, rash, bulging fontanelle, pain or injury concern also require urgent in-person pediatric assessment.

1. Disposition level 15

Is the baby definitely younger than 12 weeks, previously assessed as having uncomplicated colic, consolable, alert and content between episodes, feeding and urinating normally, with measured temperature below 38 C and none of the emergency or urgent features above?

Home comfort measures are appropriate only for a previously assessed, otherwise well, consolable baby with normal feeding, urine output, temperature, breathing, colour, and behaviour.

Advice

Emergency precautions for a critically unwell or unsafe infant

- Call Qatar emergency services on 999 for an ambulance now; do not self-drive with an unstable infant. Follow the call handler's instructions. If there is a risk someone may hurt or shake the baby, place the baby on their back in a safe empty cot, move the unsafe person away, and have a safe adult take over if available. Never shake the baby. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: baby becomes floppy or unresponsive; breathing or colour changes; risk that someone may hurt the baby.

Urgent in-person assessment for a young infant

- Arrange urgent in-person pediatric assessment now; the exact non-emergency Qatar destination requires local approval. Do not give fever medicine to delay assessment. Call 999 if the baby becomes difficult to wake, floppy, blue or grey, has breathing difficulty or a seizure, or cannot be transported safely.
- Call Back If: feeding or urine output falls; crying or behaviour changes; temperature reaches 38 C.

Controlled comfort measures for previously assessed colic

- Use the baby's established feeding plan and try holding, gentle rocking, a warm bath, or quiet background sound. Always place the baby on their back in a safe empty cot for sleep. If overwhelmed, put the baby safely in the cot, step away briefly, and ask a safe adult to take over. Never shake the baby. Re-enter urgent assessment if the baby becomes difficult to console, feeds less, has fewer wet nappies, develops vomiting, fever, abnormal breathing or colour, becomes unusually sleepy, or the cry changes.
- Call Back If: temperature reaches 38 C; feeding or wet nappies decrease; cry, breathing, colour, or alertness changes; caregiver feels at risk of losing control.

First aid

- Call Qatar emergency services on 999 for an ambulance now; do not self-drive with an unstable infant. Follow the call handler's instructions. If there is a risk someone may hurt or shake the baby, place the baby on their back in a safe empty cot, move the unsafe person away, and have a safe adult take over if available. Never shake the baby. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Colic", <https://www.nhs.uk/conditions/colic/>
- NICE NG143, "Fever in under 5s: assessment and initial management", <https://www.nice.org.uk/guidance/ng143/chapter/recommendations>
- NICE NG194, "Postnatal care" - signs and symptoms of serious illness in babies, <https://www.nice.org.uk/guidance/ng194/chapter/recommendations>
- American Academy of Pediatrics HealthyChildren.org, "Crying Baby - Before 3 Months Old", <https://www.healthychildren.org/English/tips-tools/Symptom-Checker/Pages/symptomviewer.aspx?symptom=Crying+Baby+-+Before+3+Months+Old>

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

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